**جوستاف روسي انترناشيونال - مصر (EPE) *** **طلب علاج أول مرة / متردد**

**إسم المريض رباعياً :** مرفت نبيل محمد ابراهيم
**المحافظة :** الجيزه **الحي / المدينة :** العجوزه **العنوان :** =
**رقم البطاقة :** 27301130101161 **رقم التليفون :** 01003047905 **تاريخ تسجيل الإستمارة :** 23-10-2026
**العمر(تاريخ الميلاد) :** 13-01-1973 **المهنة :** حاصل علي بكالوريوس

## الإقرار

**أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك**

**الإخصائي الاجتماعي** **المقر بما فيه**
**التوقيع:**(----------) **التوقيع:**(----------)

**التشخيص الإكلينيكي :** سرطان بالثدي (مبادرة)
**نتائج الأبحاث :** (مرفق)
**التشخيص النهائي :** سرطان بالثدي (مبادرة)
**خطة العلاج :** أبحاث أورام مسح ذري علي العظام [12966] **قسم العلاج :** الاورام
**التعليق على التشخيص :** ----------

## تقرير اللجنة الثلاثية المتخصصة

**الاسم:** د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.**
**كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.**

**تعتمد صحة البيانات المرفقة**
**رئيس اللجنة و مدير المستشفي.**

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.**

## أعضاء اللجنة

**ا.د/** ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

## قرار اللجنة

**التشخيص النهائي :** ----------
**خطة العلاج :** ----------
**المستشفي :** ----------**قسم :** ----------
**جهة العلاج :** ----------**نفقات :** ----------
**ملاحظات :** ----------

| رئيس | مدير عام | ختم |
|---|---|---|
| المجلس الطبي المتخصص | مديرية الشئون الصحية | شعار الجمهورية |

Name: مرفت نبيل محمد ابراهيم
National ID: 27301130101161

# تقرير طبي

السيد

تحية طيبة وبعد,,

| | |
|---|---|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم: | رقم طبي: |

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

| | |
|---|---|
| تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd | تاريخ وساعة الخروج : PM 01:17 05-07-2026 |

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Bone scintigraphy

| | |
|---|---|
| التاريخ : 05-07-2026 | الوقت : -- --:-- |
| | الطبيب المعالج : د/شيماء لاشين |

| | |
|---|---|
| **Patient's Name:** | **Mervat Nabil Mohammed** |
| **Date:** | **2026-06-17** |
| **Referred by:** | Prof.Dr. **no doctor** |
| **Hospital:** | **Goustave Roussey International Egypt** |

## MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

## Known case of breast CA underwent right MRM.

### I- CHEST:

- Small right middle lobe pleural based pulmonary nodule measuring 6.8 mm.
- Few bilateral scattered fine atelectatic bands.
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

### II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. Right hepatic lobe segment VII hypodense focal lesion measuring 13 x 11 mm. no dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Few sacral, right iliac bone and left femoral head lytic bone lesions.

## Opinion:

- **Small right middle lobe pulmonary nodule for follow up.**
- **Right hepatic lobe focal lesion for dynamic MR study.**
- **Few sacral, right iliac bone and left femoral head lytic bone lesions likely metastatic for bone scan.**

*Dr.sherif Wagdy*

*Dr.sherif Wagdy*

CT Scan
[[DOCTYPE:CT_SCAN]]

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

**Name : Mervat Nabil Mohammed Ibrahim** **Date : 29-06-2026 09:46**
**Branch : Dokki** **Medical No. : 405021**
**Ref.Physician Prof : Goustav Roussey Egypt**

NILE DIAGNOSTIC CENTER

## Tc-99m MDP Whole-Body Bone Scintigraphy

Comparison: Prior bone scan dated 09/07/2025
Clinical History: A 52-year-old female patient with a known history of right breast carcinoma managed by total mastectomy, adjuvant chemotherapy, and radiotherapy, currently on hormonal therapy and presents for follow-up evaluation.
Technique: Following the intravenous injection of Tc-99m MDP, anterior and posterior whole-body planar images were acquired approximately 2 hours post-injection.

**Findings:**

- Known sacrum/right SI and left femoral neck metastases show stable to minor regression.
- Old sternal lesion (retrospectively noted) similarly stable/regressive.
- No new suspicious foci.
- Benign age-appropriate degenerative spinal/joint uptake.
- Normal soft tissues and renal excretion.

**Impression:**

- Stable to regressive known skeletal metastases. No new lesions.

**Recommendation:**

- Continue follow-up.
- Consider CT/MRI of left femoral neck for cortical stability monitoring.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine



Bone Scan
[[DOCTYPE:BONE_SCAN]]

| | |
|---|---|
| **Name:** مرفت نبيل محمد ابراهيم<br>**MR. :** 3181 | |

Detailed Medical Report

| | |
|---|---|
| **السيد** | |

**تحية طيبة وبعد,,**

| | | | |
|---|---|---|---|
| **إيماء للطلب المقدم من** | | **بشأن تقرير طبي للمريض** | |
| **بقسم:** | | **رقم طبي:** | |

**ننشرف بأن نوضح فيما يلي تقريرا عن حالته**

| | | | |
|---|---|---|---|
| **تاريخ وساعة الدخول :** | | **تاريخ وساعة الخروج :** | |

**وبعد الكشف والفحوصات والمتابعة وجد انه**

**The patient has a history of metastatic breast cancer (MBC) with bone and liver involvement, classified as ER8/8 PR 5/8, HER2 negative, Ki-67 15%. The disease recurred after neoadjuvant chemotherapy on 20-06-2022, followed by right-sided mastectomy (RT MRM) and placement of YPT2 and YPN2. Subsequent management included adjuvant hormonal therapy with Femara and Zoladex, planned through September 2025, and treatment of bone and liver metastases with multiple lines of systemic therapy, including Zometa, Faslodex, and Ibrance 125 mg. Additional metastatic sites identified were pelvic bone, ovarian ablation, and a right-sided thoracic lesion noted in December 2025.**

**The patient was noted to be premenopausal with metastatic breast cancer on Faslodex and Ibrance; a CBC required for Ibrance administration showed neutropenia (ANC 0.87 ×10⁹/L), leading to postponement of the Ibrance cycle, with the medication to be received on 07-03-2026 after repeat labs.**

**Pharmacy records on 07-03-2026 indicate that laboratory results obtained on 07-03-2026 were accepted, and Ibrance 125 mg was approved for dispensing.**

**On 04-04-2026 the patient was continued on IBRANCE 125 MG plus Faslodex, with Zometa also prescribed; the next oncology visit was scheduled after repeat laboratory evaluation.**

**Imaging performed in June 2026 included a bone scan and a CT of the chest, abdomen, and pelvis, which showed no new complications; clinical decisions for the day were documented.**

**On 20-07-2026 the bone scan showed stable disease, and CT imaging demonstrated a solitary 12 × 11 mm lesion consistent with the known disease focus while on Faslodex, Ibrance 125 mg, and Zometa, with prior CTs having shown multiple bifocal hepatic lesions.**

**On 17-08-2026 the patient presented for Faslodex administration and Ibrance 125 mg; the total leukocyte count was 1.6 × 10⁹/L with neutrophils 0.8 × 10⁹/L, prompting a one-week postponement of therapy and a repeat CBC to be reviewed by pharmacy. The current plan is to repeat the CBC in one week before resuming treatment.**

**تحريرا في ,**

| | | | |
|---|---|---|---|
| **التاريخ :** | **19-09-2026** | **الوقت :** | |

**الطبيب المعالج :** dina makram

**يعتمد ,**


جمهورية مصر العربية
بطاقة تحقيق الشخصية
مرفت
نبيل محمد ابراهيم
٣١ش عفيفى مصطفى الشاهد-ش السودان
العجوزه - الجيزه
٢٧٣ ٠١ ١٣ ٠١ ٠١١ ٦١
IK3260933

DMS

تدعيم

٢٧٣٠١١٣٠١٠١١٦١ ٢٠٢٠/٠٦
حاصلة على بكالوريوس دراسات تعاونية

أنثى مسلمة متزوجة
اسامه عبدالحميد عبدالرحيم سيد احمد
١ البطاقة سارية حتى ٢٠٢٧/٠٦/٠٥



National ID · Back
[[DOCTYPE:ID_BACK]]

جونتاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصري لدعم صحة مصر

MDT FORM 19 / 8 / 2025 G

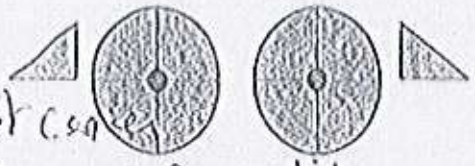
| | | | |
|---|---|---|---|
| Name | م/ امنيت سيل محمد | MRN | 0 C04/C·4 |
| | | ID | |
| Age | 52 ي. | Phone | 01003041905 |
| Menopausal status | premeno p | Marital status | ٢٧٣١١٣٠١١٦١١ |
| NRS SCORE | | Family history | |
| BMI | | REASON OF MDT | Newly develop. Bone Mets |
| CO morbidity | | | |

## Clinical Data

BX: IDC

R pS11 No visual Cri.

| ER: | PR: | HER 2: | KI67: |
|---|---|---|---|
| 8/8 | 5/8 | -ve | 15 /. |



Sonomammography: premeno p. Breast Ca with Newly developed Bone Met.

Mets Workup: Hist :- Rt Breast cancer Locally adv. TNM
L NACT --- on 20/6/2022
RrHRH ypT2 ypN2 9/15

Recommendations: Rec. adjuvant Femara + Zoladex f. 8/2022.
GU Now.
Newly Boyp :
Bone sca 9/7/2025 : Worrisome osseous lesion at Rt SI joint, sacrum, Neck of Left femur

MDT OUTCOME & PLAN Likely Metastasis.
J. HOT
HRT IST → Tabs.
Fasldex / CDK4/6 -- / V/17 - Rtx 300x10 - Zoledex

| SIGNATURE | | | | | |
|---|---|---|---|---|---|
| MEDICAL ONCOLOGY | SURGERY | RADIOLOGY | PATHOLOGY | Nutrition | Psychiatric |
| M Kelleny | Dr Hosl 19/8/2025 | | | | |

MDT Form
[[DOCTYPE:MDT_FORM]]

| | | | |
|---|---|---|---|
| Name : Mervat Nabil Mohamed Ibrahim | | Date : 18-12-2025 02:36 | |
| Branch : Portable | | Medical No. : GRI-13664 | |
| Ref.Physician Prof : | | Ins.Company : | |

## POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS:

**Findings: Right mastectomy, compared to 2/10/2025**

### I-CHEST:

Right mastectomy with clear operative bed, no masses or collection.

- Clear left breast and both axillae.
- Right upper and middle lobe subpleural reticulations likely post radiotherapy.
- Clear both lung fields.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.

### II Abdomen & pelvis:

- The liver is mildly enlarged in size with homogenous CT texture. Regressive course as regards the size of the multiple bilobar hepatic focal lesions , two are seen at segment III measures 13x11mm compared to 19 x17 mm and 8x7mm compared to 13 x6 mm in previous study, another FL seen at segment VIII measures 13x12mm compared to 22x 21 mm ,resolution of focal lesions at segment I and VI.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.

CT Scan
[[DOCTYPE:CT_SCAN]]

| | |
|---|---|
| Name : Mervat Nabil Mohamed Ibrahim | Date : 18-12-2025 02:36 |
| Branch : Portable | Medical No. : GRI-13664 |
| Ref.Physician Prof : | Ins.Company : |

- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes .
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Mild diverticular disease of sigmoid and descending colon.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.

**III The scanned skeleton:**

- **Stationary course as regards:**
- Right sacral destructive bony lesion with associated soft tissue component measures 5.5 x3 cm
- Large lytic lesion of left femoral head.
- No vertebral structural collapse.

**Opinion:**

- **Right mastectomy with clear operative bed.**
- **Regressive course as regards the size and the number of multiple bilobar hepatic focal lesions, likely metastatic.**
- **Stationary course as regards the osseous metastasis.**

*Dr. Mai Shaaban MD*



CT Scan
[[DOCTYPE:CT_SCAN]]

| | |
|---|---|
| Patient Name: Merval Nabil Mohamed Ibrahim | Date : 02/10/2025 |
| Exam : CT CAP+C (CT Chest Abd. Pelvis) | Age : 52 Year(s) |
| Medical ID: 5254/2021 | Pat. ID: 13664 |

## *POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST , ABDOMEN & PELVIS : -*

***Findings: right mastectomy***

***I-CHEST :***

**Right mastectomy with clear operative bed , no masses or collection.**

- **Prominent left level I axillary LNs.**
- **Right upper and middle lobe subpleural resticulation likely post radiotherapy**
- Clear both lung fields
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.

II Abdomen & pelvis:

- The liver is mildly enlarged in size with homogenous CT texture. Multiple bilobar hepati focal lesions , two are seen at segment III measures 19 x17 mm and 13 x6 mm , anothe FL seen at segment VIII measures 22 21 mm ,segment I measures 6 x5 mm& small lesio 10 x 5 mm seen at segment VI.they show faint contrast enhancement.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back pressure changes .
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Diverticular disease of sigmoid and descending colon.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.

***III The scanned skeleton:***

- Right sacral destructive bony lesion with associated soft tissue component measures 5.! x3 cm
- Large lytic lesion of left femoral head.
- No vertebral structural collapse .

Opinion:

- ***Right mastectomy with clear operative bed .***
- ***Right upper lobe subpleural reticulation likely post radiotherapy .***
- ***Multiple bilobar hepatic focal lesions ,likely metastatic .***
- ***Large scaral destructive lesion with soft tissue component .***
- ***Left femoral lytic lesion .***

*Dr. Reham Khedr, MI*



CT Scan
[[DOCTYPE:CT_SCAN]]

| | |
|---|---|
| Patient Name: Mervat Nabil Mohamed Ibrahim | Date : 02/10/2025 |
| Exam : CT CAP+C (CT Chest Abd. Pelvis) | Age : 52 Year(s) |
| Medical ID: 5254/2021 | Pat. ID: 13664 |

**POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS:**

**Findings: Right mastectomy**

**I-CHEST:**

**Right mastectomy with clear operative bed , no masses or collection.**

- **Prominent left level I axillary LNs.**
- **Right upper and middle lobe subpleural resticulation likely post radiotherapy.**
- Clear both lung fields.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.

**II Abdomen & pelvis:**

- The liver is mildly enlarged in size with homogenous CT texture. Multiple bilobar hepatic focal lesions , two are seen at segment III measures 19 x17 mm and 13 x6 mm , another FL seen at segment VIII measures 22 21 mm ,segment I measures 6 x5 mm& small lesion 10 x 5 mm seen at segment VI.they show faint contrast enhancement.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes .
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Diverticular disease of sigmoid and descending colon.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- No abdominal or pelvic masses or collections.

**III The scanned skeleton:**

- Right sacral destructive bony lesion with associated soft tissue component measures 5.5 x3 cm
- Large lytic lesion of left femoral head.
- No vertebral structural collapse .



CT Scan
[[DOCTYPE:CT_SCAN]]




National ID · Front
[[DOCTYPE:ID_FRONT]]




National ID · Front
[[DOCTYPE:ID_FRONT]]